CHIEF COMPLAINT: Rapidly progressive bilateral lower extremity numbness, gait ataxia, and urinary urgency over four days.

HISTORY: The patient is a 31-year-old female diagnosed with relapsing-remitting multiple sclerosis (RRMS) three years ago, currently managed on subcutaneous glatiramer acetate. Four days ago, she developed ascending tingling and paresthesias beginning in her toes that progressed up to her umbilicus. By yesterday, her legs felt 'heavy like lead,' and she required assistance to ambulate due to profound unsteadiness. She notes an electric shock-like sensation shooting down her spine into her legs upon flexing her neck (Lhermitte sign). She experienced two episodes of urinary urge incontinence yesterday. She denies fever, recent infections, visual disturbances, or facial weakness.

PHYSICAL EXAMINATION:

Mental Status: Awake, alert, fully oriented with intact cognitive functioning.

Cranial Nerves: Visual acuity 20/20 bilaterally. Pupils briskly reactive without afferent pupillary defect. Full extraocular movements without internuclear ophthalmoplegia. Facial sensation and movement symmetric.

Motor System: Normal muscle bulk throughout. Lower extremity tone is spastic bilaterally with catch at knee extension. Muscle power: Hip flexors 4/5 bilaterally, knee extensors 4/5 bilaterally, ankle dorsiflexors 4/5 bilaterally. Upper extremity strength 5/5 throughout.

Sensory: Marked sensory level to pinprick and temperature at the T10 dermatome bilaterally. Proprioception and vibration sensation severely impaired at both great toes and ankles.

Reflexes: Biceps and brachioradialis 2+ bilaterally; patellar and Achilles reflexes hyperactive (3+) with 4 beats of unsustained ankle clonus bilaterally. Babinski sign is extensor bilaterally.

Gait: Spastic, ataxic gait with wide base; unable to perform tandem walk. Positive Romberg sign.

MRI BRAIN AND CERVICOTHORACIC SPINE:

MRI Cervical and Thoracic Spine with Gadolinium: Demonstrates a focal, wedge-shaped demyelinating plaque in the posterior column of the thoracic spinal cord at the T8-T9 level exhibiting vivid peripheral ring-enhancement, consistent with active inflammatory demyelinating myelitis. Additional non-enhancing chronic plaques noted in the dorsal cervical cord at C3 and C5.

MRI Brain: Shows multiple high-signal periventricular and juxtacortical ovoid white matter lesions oriented perpendicular to the lateral ventricles (Dawson's fingers) without new enhancements.

ASSESSMENT AND RECOMMENDATIONS:

1. Acute MS relapse presenting with active acute transverse myelitis with T10 sensory level and spastic paraparesis.

2. Initiate pulse corticosteroid therapy: Intravenous methylprednisolone 1,000 mg daily for 5 consecutive days with PPI gastroprotection.

3. If no significant functional improvement after 5 days, proceed with five cycles of plasma exchange (plasmapheresis).

4. Discontinue glatiramer acetate; initiate escalation to a high-efficacy disease-modifying therapy (e.g., ocrelizumab or natalizumab) once acute flare resolves.